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Do you care or do I have a choice? Expert authority and consumer autonomy in medicine consumption

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Abstract

Consumer research has posited an ambivalent role of choice in consumer empowerment, from liberating and including to disciplining and totalising. This article provides empirical evidence for this ambivalence and nuances how consumers navigate the liminal space between expert authority and consumer autonomy.

We investigate the role of choice in consumer empowerment through an ethnographic study of medicine consumption practices in acute non-life-threatening situations in Denmark, finding these practices to be dominated by consumer choice, not only for over-the-counter but, interestingly, also for prescription-only medicine.

The implications of these findings extend beyond the context of medicine consumption. First, we provide insights on the ambivalent role of choice in consumer empowerment in a complex context of mixed commercial and public agencies. Second, we demonstrate how online information access empowers consumers to resist expert authority and act autonomously. Third, we exemplify the processes by which globalization and ideologies inform and shape consumer resistance.

Keywords: Consumer empowerment; consumer resistance; consumer choice; consumer autonomy; expert authority; health inequality.

Introduction

Nation states in Northern Europe are undergoing a neo-liberally inspired process of alteration of healthcare towards increased self-efficacy of patients (Fumagalli et al. 2015). One motivation behind this process is optimising the cost associated with professional care by having the public healthcare system shift some of the burden of care to the patient, proclaiming this change to be an improvement to patient choice (Mol 2008). Mol (2008) finds that healthcare practices are governed by two logics, a logic of (professional) care and a logic of (patient) choice.

Both public and private actors are increasingly attempting to shape healthcare by employing a consumer rhetoric (Lee 2015) that equates healthcare professionals with service providers and patients with consumers, framing healthcare as a market where consumer choice is an admissible and at times even imperative component (Shankar, Cherrier, and Canniford 2006). The phenomenon of 'patients as consumers' (Goldstein and Bowers 2015; Gusmano, Maschke, and Solomon 2019) has predominantly been studied in medical sociology, where the trope has been met with scepticism.

We will assert that the rise of the 'patients as consumers' trope is linked to larger processes of ideological change, combining ideologies of neo-liberalism and what has been termed 'healthism'. The last decades have been witness to the rise of healthism as an ideology preoccupied with 'health as a primary – often the primary – focus for the definition and the achievement of well-being' (Crawford 1980). The widespread access to medical information through the internet (Delbaere and Smith 2007) and the amplifying effect of social media on consumers' desire for choice (Scaraboto and Fischer 2013) have furthered a self-perception of individuals as responsible consumers (Giesler and Veresiu 2014) of medical services and products rather than as patients receiving medical care.

The duality of patients as consumers opens up for applying consumer research to the context of healthcare. Moreover, and more importantly, it also has the potential to provide new insights on consumption in complex contexts of mixed commercial and public agencies. The healthcare market represents such a complex context and has traditionally not been considered as choice-oriented: chronically ill 'patients seem not to embrace choice' (Salmon and Hall 2004), and 'an individual's use of the healthcare system' in acute life-threatening situations 'is likely to be involuntary and, in this sense,

necessary', resulting 'in an almost powerless buyer' (Goldstein and Bowers 2015). In contrast, in acute non-life-threatening medical situations, while care remains an expectation, consumer choice can be exercised and facilitate consumer empowerment (Schneider-Kamp and Askegaard 2020).

Consumer research has posited an ambivalent role of choice in consumer empowerment. Shankar, Cherrier, and Canniford (2006) argue that liberation and disciplining are two sides of the same coin. Scaraboto and Fischer (2013) identify consumer choice with increased market inclusion while Yngfalk (2016) points out its role in the totalising aspects of consumerism. This article provides empirical evidence for this ambivalence and nuances how consumers navigate the liminal space between expert authority and consumer autonomy.

Concretely, we investigate the role of choice in consumer empowerment by studying individuals' practices of medicine consumption, both when they interact with healthcare professionals and when they act independently. Here, by medicine consumption we refer to a set of practices including, but not limited to, evaluating, choosing, obtaining, storing, and applying over-the-counter (OTC) and prescription-only medicine (POM). Through an ethnographic study, we aim to *investigate the logics governing these practices as they unfold in the lived experiences of individuals faced with acute non-life-threatening medical situations.*

We find a dominant role for a logic of consumer choice for both OTC and POM and demonstrate how access to online information supports individuals in expanding both the circumstances and the range of their consumer choices. The implications of these findings extend beyond the context of medicine consumption. First, we provide insights on and nuance the ambivalent role of choice in consumer empowerment in a complex context of mixed commercial and public agencies. Second, we demonstrate how online information access empowers consumers to resist expert authority and act autonomously. Third, we exemplify the processes by which globalization and ideologies inform and shape consumer resistance in a context of mixed commercial and public agencies.

Theoretical background

From the second half of the 20th century, we have witnessed a profound change in the role of patients in healthcare, away from the traditional role of being 'passive recipients of doctors' orders' (Conrad 1985, 31). Already Szasz and Hollender (1956) introduce a typology of patients: passive patients, patients that can enter into a dialogue, and patients that collaborate. Watts (1975, 498) discusses to what extent patients could and should be viewed as consumers with freedom of choice as part of the discourse on the then rather new concept informed consent, where patients are given 'a decisive role in determining what will or will not be done'. Tuckett et al. (1985) take this development a leap forward, by even viewing doctor-patient interactions as meetings between experts.

On this backdrop, Mol (2008, 55) observes that 'doctoring is not something that only doctors do' and introduces the concept of active patients, i.e., patients that (partly) take over their healthcare in close collaboration with healthcare professionals. According to Mol (2008), healthcare practices are governed by two logics: a logic of care based on professional healthcare, and a logic of choice based on the idea of patient choice. In the remainder of the article, we will refer to her logics as 'logic of (professional) care' and 'logic of (patient) choice'. Here, patient choice is the legitimate continuation of informed consent, with the final right of medical decision-making firmly placed with the patient. In Mol's (2008, 8) terminology, a logic denotes the rationale behind which practices are considered 'appropriate or logical to do in some site or situation, and what is not'.

Mol (2008, 1) also observes that '"patient choice" and "good care" may sometimes complement each other, but more often they clash'. She argues that 'practices designed to foster "patient choice" erode existing practices that were established to ensure "good care"'. Rather than patient choice, the focus of healthcare professionals is predominantly on good care based on high levels of safety and quality of treatments (Allen et al. 2016). The backside to this has been a tendency to a professional closure around biomedical perspectives that may be quite inert with respect to including the patient's experiences and feelings in the treatment. The rise of the logic of choice should be seen in the light of neo-liberal forms of "empowerment through the market" but also as a reaction to this ineptitude.

In contrast, then, and not surprisingly, policymakers are increasingly focused on logics of choice including 'choice of any provider, choice of consultant-led team, choice of GP practice and choice of treatment' (Department of Health 2012) in line with a more general political agenda focusing on increased competition on the market, transparency, and citizens' freedom of choice. Here, the term choice encompasses not only the narrow concept of patient choice but also the broader concept of consumer choice (Bettman, Luce, and Payne 1998). While the former limits patients to choose from a number of alternatives proposed by healthcare professionals or reject them, the latter allows for a more active stance, where patients as consumers actively choose and combine service providers, treatments, and medical products (Geiger and Prothero 2007).

Henderson and Petersen (2002) describe this transformation of healthcare from a purely medical good monopolised by healthcare professionals towards a commoditised good on the global market as driven by consumer logics and narratives. Such narratives have been explored by Geiger and Finch (2016) in the context of the market for pharmaceutical products, where they were found to shape market actors' perception of patients as active consumers.

There are many labels for the more active role of patients as consumers on the healthcare market. Fumagalli et al. (2015) review the related concepts empowerment, involvement, activation, engagement, enablement, and participation in their meta-study of patient empowerment. Cova and Dalli (2009) reflect more generally upon consumers' active role by employing concepts such as resistance, empowerment, agency, co-production, and collaboration, echoing Denegri-Knott et al. (2006, 958) who open up to viewing 'consumption as site of resistance, theorizing consumers as creative and playful agents' counteracting 'the strategic manoeuvres of powerful corporate players'.

Izberk-Bilgin (2010) theoretically reflects on consumer resistance through the tension spanned by the enslavement/manipulation and the agency/empowerment discourses, summarizing marketing and consumer research on resistance and pointing out the need for further research on how resistance is informed and shaped by globalization as well as by (religious) ideologies. Scaraboto and Fischer (2013, 1254) demonstrate how consumer choice empowers 'plus-sized' consumers on the free market of fashion, pointing out the need for future research in 'contexts where actors who are less culturally

stigmatized try to influence particular organizational fields'. Healthcare constitutes such a context, where the power that consumers are resisting to is predominantly based on (medical) authority.

Abercrombie (1994, 41) defines authority as a 'sub-type of power in which people willingly obey commands because they see the exercise of power as legitimate'. Authority in small-scale authority contexts such as the relationship between healthcare professionals and individuals is then often based on one or more of the following four supports: superior *expertise* of the superordinate, feelings of *deference* of the subordinate, feeling that the boundary between superordinate and subordinate is *taboo*, or a domination of *meaning* by the superordinate.

Abercrombie (1994, 44) argues that resistance 'may take a different form for each of the four supports of authority'. As an example, 'the authority-through-expertise of the doctor might be contested actively by the development of alternative medical practices or, more passively, by the use of common sense or traditional practices and remedies'. For consumers to resist medical authority by challenging the medical expertise of formally trained healthcare professionals, they need access to 'alternative bodies of knowledge'.

Such alternative bodies of knowledge have recently become widely available through the Internet and the ensuing ubiquitous online access to health-related information and increase in health literacy (Estacio, Whittle, and Protheroe 2019). Ouschan et al. (2000, 100) connect access to such information with consumer resistance by observing that 'consumerist behaviour is only likely to occur when the patient feels empowered to question the doctor, make independent judgments or seek out alternative sources of information'. Weaver et al. (2010) already found health information-seeking behaviours to be wide-spread, and Goldstein and Bowers (2015, 164) identify patients that act as consumers with 'the more educated, involved individual with access to informational resources'.

The phenomenon of consumer resistance is thus complicated not only by the type of authority at stake with medical professionals (compared to, for example, fashion professionals) but also by the tension between logics of care and logics of choice. Consequently, in this context, we are mainly inspired by Shankar, Cherrier, and

Canniford (2006, 1014) who employ a Foucauldian analysis to demonstrate that 'consumers can be empowered or liberated through exercising choice but that choice is also disciplining and potentially paralyzing'.

This ambivalent role of consumer choice can be understood as a critique of the liberal humanist notion of consumer empowerment. On one hand, some consumers create 'privileged spaces' that allow them to transcend the 'imperatives and models of discipline imposed' (Shankar, Cherrier, and Canniford 2006, 1019). On the other hand, the exertion of governmentality enables the 'authoritative realm' to learn about and forge 'self-disciplined consumers' through 'advertising and promotional discourses' (Shankar, Cherrier, and Canniford 2006, 1018).

The work on agentic consumer resistance cited above, as well as Shankar, Cherrier, and Canniford's (2006) analysis assume 'free', commercial markets as a context. In a more complex context of mixed commercial and public agencies, consumer narratives have been found to be erosive to medical authority, with trust shifting from public and commercial 'health product and service providers' to 'peers and friends' as well as 'alternative practitioners' (Kristensen, Lim, and Askegaard 2016). This erosion of authority clashes with policymakers' and healthcare professionals' interests in the responsibilisation of health consumers to optimise the efficiency and cost of treatment of their citizens and to ensure compliance among patients (Lupton 2013a).

Context

The context of the study is the Danish egalitarian welfare state, where 'welfare benefits and services are financed primarily by taxes' (Christiansen and Markkola 2006). The Danish public health system is built around the right for free treatment by general practitioners and hospitals in the case of illness. Appointments to general practitioners can be obtained the same day for acute conditions and within a few days to a couple of weeks otherwise. For specialist help, there is a treatment guarantee within 30 days, which is lowered to 14 days in the case of life-threatening conditions such as cancer.

General practitioners are the main providers of primary care, being specialists in diagnosing and treating the most common conditions and diseases as well as taking care

of routine consultations regarding paediatric and obstetric conditions. Most medical specialists can be consulted free of charge after a referral from general practitioners. Outside of regular opening hours, the Danish regions operates urgent care centres as well as out-of-hours medical services, often located at larger hospitals.

Danish consumers can purchase two types of medicine: over-the-counter (OTC), for which they pay the full market price, and prescription-only medicine (POM), which is subsidised by the state. Subsidy rates differ according to yearly individual spending on medicine, ranging from 0% for the first USD 150 to 100%, ensuring that individuals never pay more than USD 600 per year. For both types of medicine, pharmacists are required to provide advice and opportunities to obtain cheaper generic variants.

Only general practitioners and medical specialists may issue prescriptions, which are registered in a national-level electronic database accessible to pharmacies. The distribution of POM is strictly limited to pharmacies, and retail prices are fixed every 14 days on a national level. Even in the light of state subsidies, nearly one out of ten prescriptions end up being not filled (Pottegård et al. 2014).

While consumers can freely choose and purchase OTC medicine, it is still regulated with respect to which active ingredients are allowed, which producers are licensed, how it may be marketed, which distribution channels are sanctioned, and how the market players construct retail prices. Most OTC medicine is only available through pharmacies to ensure expert advice, with only the most basic medicine such as weaker painkillers and antihistamines available in limited package sizes through supermarkets.

The rather high level of regulation of both types of medicine simplifies as well as complexifies some of the usual steps of the buyer decision-making process as commonly applied for FMCGs (Kotler 2008). As our interest in medicine consumption extends beyond the decision-making process, we include the following practices of medicine consumption: evaluating (finding information on medicine; evaluating this information), choosing (evaluating alternatives; decision-making), obtaining (obtaining a prescription, if needed; purchasing the medicine through a distribution channel), storing (stock-piling medicine for potential later use), and applying (ingesting; administering; injecting).

Methods

The qualitative data for this study was collected through ethnographic methods in the period from 2014 to 2017 relying on a sample of 22 informants. Non-probabilistic network sampling (Bernard 2011) was employed to recruit informants through the snowball method. Network sampling allows for building upon existing trust relations, easing access to informants willing to be studied regarding an intimate and sensitive topic such as health-oriented practices (Bell et al. 2017). The first author recruited the pool of initial informants through their secondary networks, avoiding inherent biases when dealing with members of the primary network. At most, two referrals per informant were included, excluding those without recent experience of buying OTC and/or POM as well as physically or cognitively severely incapacitated individuals.

Recruiting was a continuous process where data collection from already recruited informants was interleaved with the recruitment of further informants. After interviews with 14 informants were conducted, the additional data collected to a large degree supported rather than extended existing categorizations, indicating 'theoretical saturation' (Glaser and Strauss 2009). The actual recruited sample consisted of the 22 informants listed in **Table 1**. These numbers are well-aligned with expectations for obtaining high confidence categorization results from qualitative interview data (Rowlands, Waddell, and McKenna 2016).

Table 1. Informants

Pseudonym	Gender	Age	Occupation	Interviews no. (hours)
Andreas	male	26	shop assistant	2 (4½h)
Clara	female	34	personal shopper	2 (5½h)
Daniel	male	27	student	2 (3h)
Ditte	female	46	secretary	2 (3h)
Henrik	male	57	technician	3 (4h)
Jan	male	34	researcher	1 (2h)
Johannes	male	29	business consultant	4 (7½h)

John	male	52	communications engineer	1 (3h)
Katrine	female	34	teacher	4 (8h)
Kirsten	female	63	retired	3 (4½h)
Lars	male	38	designer	2 (3½h)
Mads	male	25	student	2 (5½h)
Margit	female	51	civil servant	3 (7h)
Martin	male	33	marketing coordinator	2 (2½h)
Mia	female	43	software tester	4 (7½h)
Nils	male	31	IT consultant	3 (4½h)
Ole	male	54	unemployed	2 (3h)
Per	male	38	entrepreneur	2 (4½h)
Rebecca	female	41	nurse	2 (4h)
Rolf	male	59	police officer	1 (2½h)
Siri	female	27	student	3 (3h)
Vibeke	female	39	project manager	3 (4½h)

The overall research design follows the basic idea of ‘shopping with consumers (SWC)’ (Lowrey, Otnes, and McGrath 2005) but had to be adapted in several points. As in SWC, we first contacted potential informants and checked their suitability. The first author then established rapport through initial visits to informants’ homes, which constituted a natural setting for many of the medicine consumption practices of interest and provided a place of emotional comfort. The visits included long, in-depth open-ended interviews (McCracken 1988) lasting at least two hours each. The first author visually documented informants’ medicine portfolio (any stored products that they identified as ‘medicine’), when permissible, and made individual agreements about how informants would be followed in the future.

Unlike the scheduled shopping trips in SWC, acute non-life-threatening medical situations arise unpredictably. The first author explained the research design to informants, carefully asking if they would agree to be accompanied to consultations with general practitioners or the local pharmacy when an acute situation would arise. While a few informants had reservations to be accompanied and observed in such private situations, most agreed to contact the first author in such situations. These agreements yielded a total of 10 accompanied purchases (Mariampolski 2005) to local pharmacies, where the interactions between informants and pharmacists were observed, both when OTC medicine were purchased and when prescriptions for POM were filled. In four cases, informants also invited the first author to accompany them during their interactions with general practitioners or medical doctors at the out-of-hours medical service.

Accompanied visits to pharmacies and general practitioners were followed by in-depth interviews, typically after a gap of one to two weeks. In addition, frequent contact with informants over extended periods of time was kept through e-mails, phone calls, social media, and in a few cases spontaneous meetings. For 19 of the 22 informants, keeping in contact eventually resulted in one or more follow-up interviews – see **Table 1** for details –, which were conducted fully open and unstructured, minimising any formal interaction with the informants. Here, also the decision-making processes during the accompanied purchases and visits were discussed, where applicable.

The primary data, therefore, consist of 53 interview recordings (approx. 97 hours), 37 photographs, and 84 single-spaced pages of field notes regarding both observations and reflections. The first author performed and recorded all interviews and transcribed and translated them to English, striving for adequate and equivalent translations, with the goal of keeping the meaning as intact as possible.

The photographs only show the objects the informants identified as medicine and never persons or their homes, as they are intended as ‘scientific documentation’ rather than as a means of studying the ‘visual culture’ (Pink 2006) of the informants. We presented and anonymously discussed the contents of some of the medical portfolios with health professionals in order to gain a better understanding of their contents.

The field notes can be divided into three types: initial impressions and observations from home visits and interviews, field notes during non-participant

observations, and further reflections on both interactions and observations in the time following (Davies 2008). The field notes and photographs were jointly used to support the analysis of the interview data based on the idea of triangulation (Denzin 1978). The first author obtained verbal consent for producing recordings and photographs as well as for publishing relevant non-identifiable excerpts from interview transcriptions. For de-identification, we assigned pseudonyms to all informants and broadened very specific occupations carefully.

We coded the data in two stages. First, we used open coding to inductively code, conceptualise, and categorise the data (Corbin and Strauss 2014), obtaining four main categories: *knowledge* with subcategories *information seeking*, *expertise* and *medical terminology*; *decision-making* with subcategories *authority*, *resistance*, and *compliance*; *medicine* with subcategories *OTC*, *POM*, *brand*, *generics*, and *alternative*; and *emotional involvement* with subcategories *feelings*, *perceptions*, and *beliefs*.

Second, we introduced a deductive element to the process by adding codes and categories inspired by Mol's (2008) dichotomy between logics of care and choice in order to highlight the ambivalent role of choice in consumer empowerment in a complex context of mixed commercial and public agencies as well as categories and codes based on Abercrombie's (1994) four supports of authority to investigate how consumers are empowered to resist expert authority. We recoded the data and discovered codes relevant for the following new categories: *care,* with subcategories *professional healthcare* and *self-care*; *choice,* with subcategories *patient choice* and *consumer choice*; and *expertise,* with subcategories *professional expertise* and *counter-expertise*.

Findings

The findings are structured along with the distinction between OTC and POM and investigate the logics governing the practices of medicine consumption by uncovering the construction of consumer choice in the relevant authority contexts between consumers and healthcare professionals.

Consuming OTC medicine: professional care vs consumer choice

The relevant authority context (Abercrombie 1994) for the consumption of OTC medicine is the one between the pharmacist as superordinate and the consumer as subordinate. Pharmacists support the consumer's purchase by offering professional advice on drug selection, safety, and dosing. Their authority is supported by their expertise.

The expertise of pharmacists is challenged by consumers such as 31-years old Nils, who reflected on the role pharmacists play in his choice of medicine:

> I feel like I don't necessarily need someone in the pharmacy to tell me what to choose. I already know: I need Losec® [medicine for treating acid reflux based on the proton pump inhibitor omeprazole]. Maybe the pharmacist can make recommendations regarding the dosage.

Nils openly renounces a central role for professional care, delegating at most an informative role regarding matters of drug dosing to the pharmacist. He negates a more general need and role for professional advice when purchasing OTC medicine, relying on freely available health-related information and the 'collective intelligence' of Internet peers (Lefebvre 2007, 33) for his choice of OTC medicine instead:

> We [Nils' network] call it OSINT: Open-source intelligence. One just reads up on the medicine, sees what the side effects are, how it works. Everything is available on the Internet.

Having access to other bodies of knowledge enables Nils to resist the authority-through-expertise (Abercrombie 1994) of the pharmacist in his consumer choice of OTC medicine. The Internet facilitates such health information-seeking behaviours (Weaver et al. 2010), allowing consumers to exercise consumer choice rather than rely on professional care.

Interestingly, such behaviours also have the potential to foster the construction of consumer choice through brand commitment. Faced with a stomach ulcer, Nils had chosen a certain OTC medicine brand (Losec®) to be the solution to his plight:

> I need Losec®. Don't even try to offer me some generic knock-off.

The field notes summarize the explanation for this clearly articulated brand commitment: 'NB: Nils chooses exclusively by online research (others' experiences and recommendations)'. Nils, thus, constructs his consumer choice through relying on the choices of online peers rather than by traditional brand awareness measures such as advertising.

John, 52-years old, demonstrated how consumer choice has the potential to immediately yield resistance to professional recommendations when he was faced with a sore throat:

> When I go to a pharmacy and want to buy Zyx® [a benzydamine-based sore throat remedy], even when the pharmacist recommends Coldyx® [generic benzydamine], I am not going to abandon Zyx®. I am still going to buy it and not the idiotic thing the pharmacist recommended.

From further discussion with John, it becomes clear that he was not aware that pharmacists are legally required to optimise the patient's resource allocation by offering cheaper generic variants. John seemed antagonised by perceiving the recommendation of the pharmacist as an attempt to project their authority to restrict consumer choice. Similar to Nils, John underlined that he constructs his choice through knowledge built from online research and Internet-based user reviews rather than relying on the expertise of pharmacists.

These two cases are indicative of a seemingly common mechanism: consumer choice constructed through access to online information has the potential to challenge the professional expertise in the authority context between consumers and pharmacists, empowering consumers to resist the medical authority built upon this expertise. Such resistance, ultimately, contributes to demoting the role of pharmacists to that of basic merchants.

In contrast, health-information seeking behaviours can also foster awareness about generic brands and the distinction between brand names and active ingredients of

OTC medicine. Case in point, consider 27-year old Daniel, reflecting on his purchasing of painkillers:

> Ibuprofen® [generic ibuprofen] is the same thing as Ipren® [ibuprofen-based painkiller], just cheaper. So, actually, I buy Ibuprofen®, but I call it Ipren®. It's a bit weird!

Daniel ordered medicine using an established brand name, even when expecting to purchase a generic variant. The use of brand names for a generic category of products is a well-known phenomenon referred to as genericisation (Cova and Paranque 2016). In further discussion, Daniel explained that he expected to be offered the cheapest variant of the medicine regardless of how he referred to it. Several informants expressed the expectation of such a mutual default understanding between pharmacists and consumers about price as the dominant factor for constructing consumer choice of OTC medicine.

While Daniel's focus on price is implicit and mediated through the pharmacist, 39-year old Vibeke takes a more explicit stance on the topic:

> If I have a choice, I will go for the cheapest medicine. Price! I don't care for the brand. If I know the [active] ingredients are the same, why should I pay more?

Her awareness and understanding of the concept of active ingredients in pharmaceutical products empower her to resist the marketing power of established pharmaceutical brands and to exercise informed consumer choice instead.

Vibeke's practices of medicine consumption were not limited to evaluating, choosing, and purchasing medicine but also include storage and maintenance, as Vibeke pointed out while laughingly sorting through her collection of medicine kept in some drawers in her bedroom as well as on a shelf in her kitchen:

> I have much more. It is in my bedroom; I have a special shelf for medicine. I do not use all of it. I'm actually buying, keeping, and then ... out [pointing to the waste bin].



Figure 1. Medical portfolio of 39-year old Vibeke (no chronic medical conditions).

Vibeke's sizable medicine portfolio is documented in **Figure 1**. Interestingly, most informants were found to store and manage large amounts and varieties of medicine at home. None of the informants with large medicine portfolios had any chronic or acute medical conditions, i.e., they had neither a constant nor an immediate need for medication during the period of the data collection.

These rather sizable medicine portfolios imply either a reckless application of medicine or a disparity between the amounts purchased and the amounts applied. The actual needs for medication for these informants seemed to revolve at most around the limited ad-hoc application of painkillers and other common cold remedies. What then are their reasons for such extensive medicine consumption?

The first author discussed the medicine portfolios in detail during the research visits, finding that the informants knew and understood the purpose of all or nearly all the medical products stored. When asked about random items in their portfolio, the informants usually not only knew the purpose of the medicine, they also knew the main active ingredients, as well as details about why, where, and for what price the medicine was purchased.



Figure 2. Medical portfolio of 29-year old Johannes (no chronic medical conditions).

Asked directly about his reasons for storing such a large amount of medicine at home as documented in **Figure 2**, 29-year old Johannes answered:

> If something is wrong with me, like there is some problem with my skin, or I have some kind of cold, I ask myself what it could be and what could help with this. Does it make sense to go to a doctor or can I do something about it at home with the help of my medicine?

Johannes stored medicine as a means of expanding the range of his consumer choice in acute non-life-threatening situations. Storing medicine allowed him to choose whether to consult healthcare professionals or to perform self-care in cases where he felt empowered to diagnose and treat himself.

To summarise, most informants actively sought information on OTC medicine online, evaluated this information to choose which medicine to purchase, and stored and maintained sizable medicine portfolios at home. We found a dominating role for a logic

of consumer choice such constructed between brand commitment and price-oriented purchasing. On the one hand, we demonstrated that consumer choice can be liberating in the sense that it empowers consumers to resist the medical authority of healthcare professionals or the marketing power of pharmaceutical companies. On the other hand, one might argue that consumer choice has a paralysing effect on the more brand committed consumers while the promotion of generic variants of medicine forges self-disciplined consumers.

Prescription and choice: a love-hate relationship

The relevant authority context (Abercrombie 1994) here is between the medical doctor as the superordinate and the consumer as the subordinate, with pharmacists having a supporting role regarding controlled delivery of the medicine and information about generics. The authority of medical doctors is primarily supported by their expertise and secondarily by a combination of feelings of deference on the consumer's side as well as a domination of meaning on the medical doctor's side.

On first sight, the highly regulated chain of prescription from medical doctors via pharmacists to consumers lacks the freedom of choice and, consequently, the exposure to a consumption approach based on the logic of consumer choice as seen in the preceding section on OTC medicine. The choice seems to be limited to selecting between functionally identical generic variants. In contrast, the logic of (professional) care seems to be dominant for POM, with medical doctors in the exclusive role of prescribing the medicine and pharmacists controlling the delivery.

The traditional perspective on this chain of prescription and the expectations regarding a passive recipient-of-care role of the consumer were summarised by 63-year old Kirsten when she was asked what happened after she received a prescription from her general practitioner:

> I went to the pharmacy, got my medicine, and took the pills.

This linear process, however, is not a necessity. A closer look at our informants reveals that practices regarding POM also are subject to a logic of consumer choice similar to the

one uncovered for OTC medicine.

We observed practices indicating a strong role for consumer choice in the case of antibiotics, which are one of the most prolific classes of POM in the Danish context. Denmark holds the questionable fame of having the highest level of antibiotics prescriptions among the Nordic countries (Larsen 2015). State-initiated informational campaigns in Denmark have therefore stressed that antibiotics consumption should be limited to necessary cases but carried out consequently when prescribed. The need for consequence is explained by 38-year old Lars:

> I'm worried about antibiotics because it's in my head that you don't stop taking antibiotics. If you stop, your body builds a resistance. And then they [antiobiotics] do not work anymore.

From further discussion with Lars, it becomes apparent that the informational campaigns on responsible antibiotics consumption have given him an understanding that one is expected not to discontinue a course of antibiotics once started to avoid bacterial strains from developing resistance to antibiotics.

Consumer scepticism towards antibiotics is reinforced by an awareness of their side effects and their ineffectiveness regarding the treatment of viral infections. This scepticism fosters consumer resistance towards the authority of medical doctors among some of the informants. This resistance can occur both in the open, e.g. when the necessity of prescribed medicine is discussed with medical doctors, and in the hidden, e.g. by accepting the prescription, but either not buying the drug or even buying it, but never actually applying it.

A case of consumer resistance in the open was reported by 38-year old Per when he described a visit to the family's general practitioner with his 1-year old son:

> We came to our doctor with our son. He had a high fever for several days. Instead of the kids' usual doctor, there was a young doctor, probably in training. She immediately prescribed a course of antibiotics – without even making a blood test!

The field notes unpack what Per refers to as a blood test: 'blood test = CRP test for

inflammation level', i.e., a test for the level of C-reactive proteins, which is an indicator for systemic inflammation levels. Higher inflammation levels often correlate with bacterial infections rather than viral ones.

Instead of deferring to the general practitioner, Per repeatedly questioned the prescription and thereby the underlying logic of (professional) care. When his attempts failed to sway the general practitioner's position, and he still felt unsatisfied with the given explanations, Per escalated the conflict:

> I was so irritated! I insisted that our usual doctor gives a second opinion and that a blood test is made. Turns out, antibiotics were not necessary, as often. Good for us, but is everyone prepared to fight for their children in this way?

When stating that antibiotics were unnecessary, Per refers to his son's CRP levels being low and, thus, a low probability of a bacterial infection treatable by antibiotics,

While Per was feeling empowered to openly resist the general practitioner's authority, his assumption that not everyone is willing and able to resist in this way is confirmed by 34-year old Katrine, who did not openly oppose the decision-making of a general practitioner at the out-of-hours medical service when she received a prescription for antibiotics for her 4-year old daughter:

> We had the same problem several times before and it was never treated by antibiotics. I was so surprised. Negatively! We were never giving her [the daughter] antibiotics. So, we went to the pharmacy and bought these antibiotics.

Based on her previous experience with a similar presentation of symptoms, Katrine did not consider the application of antibiotics to be necessary. After obtaining the antibiotics, she attempted to verify the adequacy of the prescription by seeking out officially-sanctioned health information made available by the Danish health ministry:

> They [health authorities] write that viral infections are the most common cause. And recommend a wait-and-see approach in such cases.

A wait-and-see approach refers to monitoring the development of the symptoms for a number of days before starting the treatment and not starting the treatment at all if the symptoms and the general health state of the patient improve. Katrine also referred to the rise of antibiotics resistance, questioning the choice of the particular type of antibiotics prescribed:

> But you actually never can be sure that antibiotics can help even with bacterial infections. There are so many bacteria now, and some have become resistant to different types of antibiotics. The doctor prescribed us some antibiotics, which they [database on recommended medicine usage] say should be used as a second line of defence.

Having access to high-quality online health information empowered Katrine to question the expertise and experience of healthcare professionals. As an outcome, she resisted their authority and decided to apply a wait-and-see approach instead of applying the prescribed antibiotics immediately:

> So, we actually never gave her [the daughter] the antibiotics. We bought them, kept them around for a while, and later threw them out.

Katrine's choice to delay the application of the antibiotics, thus, challenged the traditional chain of prescription (medicine is prescribed, bought, and applied as instructed). In this case, the general practitioner still wrote a prescription, the pharmacist still delivered the POM, and the Danish state still subsidised the purchase. Nevertheless, for the sick body, there was no effect, as the medicine was never applied as intended. And the antibiotics probably even ended up contributing to the development of resistant bacterial strains as they ended up in the landfill or the waterways instead of being delivered to the pharmacy and disposed of correctly. The effect of such consumer resistance in the hidden is opaque to medical doctors, who have no choice but to assume that filling the prescription is equal to the treatment having been performed, i.e., to the POM having been applied.

While (Mol 2008, 44) in the context of chronic disease management would likely argue that no treatment cannot be a choice, for acute non-life-threatening situations no treatment indeed can be a choice. This argument holds both for not filling the prescription

and for filling the prescription but delaying (maybe indefinitely) the treatment using a wait-and-see approach as demonstrated above.

It is questionable, though, whether either following a prescription or performing no treatment at all provides sufficient grounds for 'truly sovereign consumer choices' (Denegri-Knott, Zwick, and Schroeder 2006, 956), and thus can be attributed to a logic of consumer choice. In the following, though, we demonstrate that consumers expand the circumstances and range of their consumer choices beyond accepting or rejecting suggested treatments.

The first observation is that no POM does not equal no treatment. Mia, 43 years old, was challenging the authority-through-expertise of her general practitioner by resorting to alternative medical practices (Abercrombie 1994). Her main health concerns were with being a vegan and the influences this fact had on her immune system:

> I am a vegan. I know that keeping this diet means my body is lacking a lot of nutrition. And that makes my body sick. But I never would treat my body with chemicals. Only natural supplements! I use only natural medicine. And it improved my immunity to be able to deal with all the diseases I potentially could get.

The field notes identify Mia's 'chemicals' as 'non-herbal medicine', i.e., traditional Western medicine and her 'natural medicine' as 'herbal medicine as well as nutritional supplements'.

Mia was exposed to (at times) contradictory ideals of responsible consumption, e.g. when the moralistic imperatives of healthism and veganism clash. Giesler and Veresiu (2014) describe such clashes in the context of 'the many increasingly complex moralised landscapes of choice options, prescriptions, treatment regimes, recommendations, and prohibitions that consumers are expected to navigate today'.

Mia was, in general, pre-occupied by and conscious about her health, leading her to frequently consult her general practitioner. Asked to recall an acute episode that led to a prescription of medicine, she recounted the following experience:



Figure 3. Medical portfolio of 43-year old Mia (no chronic medical conditions).

> Once I was having a lot of headaches, but my doctor could not figure out why I got them. I started to read up on it, and I read on the Internet that high doses of vitamin C prevent headaches and help to treat the ones you already have. I started taking a very high dose of vitamin C.

The field notes detail that Mia received, amongst others, prescriptions for migraine medicine but never filled them and instead resorted to alternative treatments. In another situation, where she was sick after having the flu and other cold viruses, she chose to ingest a course of vitamin C so highly dosed that she observed stomach problems due to the acid and her skin turning yellow.

Mia's alternative medicine consumption is centred around vitamins, minerals, and a selection of natural supplements as documented in **Figure 3**. No other medicine was found to be stored at her home. When choosing OTC supplements instead of the POM prescribed by her general practitioner, Mia clearly relied on a logic of consumer choice

that empowers her to resist the authority of her general practitioner by challenging their expertise with the help of alternative bodies of knowledge (Abercrombie 1994), both in the form of medical literature and online sources of health information.

The logic of consumer choice applied to POM has so far empowered consumers to break the chain of prescription at the point of purchase or application, replacing the POM by no or an alternative treatment. Some consumers, though, feel empowered to decide on their prescriptions independent of general practitioners. Henrik, 57 years old, described how he obtained the POM that he chose himself:

> I'm talking to him [general practitioner] one time per year or less shortly by email and ask him to send me some prescriptions. And, actually, I don't need a doctor – just an email and the medicine from the pharmacy. Done.

Henrik does not consider the expertise of general practitioners, nor does he see a point in showing deference or leaving meaning-making up to them.

Likewise, 29-years old Johannes performs the choice of the POM independent from his general practitioner:

> Only if I don't have the right medicine at home, I will contact the doctor and he can prescribe me that medicine.

Both Henrik and Johannes deconstruct the authority of general practitioners, mainly utilising them as a formal way of gaining access to POM. Their approaches to expanding their consumer choice to POM are likely not universally applicable. It requires a confident consumer that feels empowered to demand prescriptions from general practitioners by email in combination with access to general practitioners who are willing to issue prescriptions without consultations.

Other consumers found alternative ways to exercise consumer choice regarding POM, though. In particular, several informants actively diversified their purchasing options by buying medicine abroad. Vibeke's medical portfolio (**Figure 1**) contained some medicine that was either unavailable or classified as prescription-only in Denmark:

> This is one, anti-flu, Tamiflu®, you cannot buy it in Denmark. This is my English pill, it is very good if you have a headache. I always buy it in London, or Søren [her husband] brings it. This is from the US, it is a very good painkiller cream, you cannot find it anywhere [in Europe]. This is a Greek one, good as well.

Both Tamiflu®, the 'English pill', and the 'Greek one' were identified to be POM in the Danish context. Vibeke obviously valued effectiveness and availability but does not seem to be concerned with Danish regulations. She constructed her consumer choice of POM based on the same price-oriented purchasing she applied for OTC medicine:

> I know those products [medicine classified as prescription-only] are much cheaper than here in Denmark. The same product is 20 times more expensive here.

Vibeke's consumer choices were only price-oriented, though, as long as she believed the products were identical in effect on her body:

> But if I need my American cream – I will buy it whatever it costs. And by the way, it is the cheapest of this type of medicine – believe me! I have checked it in the US.

The possibility to purchase medicine abroad empowers Vibeke to exercise freedom of choice even for products not freely purchasable in Denmark. Another informant constructing his consumer choice on the international marketplace is Johannes, whose medical portfolio (**Figure 2**) contains medicine from a broad variety of Eurasian countries. Vibeke and Johannes' expand the range of their consumer choices by creatively circumventing the regulations of the national health markets. Just as in the case of medical tourism (Perfetto and Dholakia 2010), such consumer freedom is a double-edged sword, though, further enabling practices of autonomous self-treatment widely considered as undesirable and outright dangerous among health professionals.

To summarise, consumers have the choice of breaking the chain of prescription at any point. Just as for OTC medicine, the consumption of POM as a buying process is separated from its actual physical consumption, e.g. its application by ingestion. Consumers also rely on access to online information to expand their choices regarding POM, including consumer choices such as no treatment, alternative medicine, and the

purchase of medicine abroad. Also, for POM, we found that a logic of consumer choice dominated practices of medicine consumption, with the logic of (professional) care supplied by medical doctors as experts in diagnosis and treatment selection often playing an insignificant role. On one hand, consumer choice was found to empower consumers to resist the authority of medical doctors and foster autonomous self-care even in the highly-regulated situations surrounding the prescription of medicine. On the other hand, we demonstrated how the ideology of healthism and promotional campaigns regarding antibiotics overuse forge self-disciplined consumers.

Discussion

We have already discussed how, in the context of chronic disease management, Mol (2008) juxtaposes the logic of (professional) care with the logic of (patient) choice, questioning the appropriateness of the latter and arguing for strengthening the former. Mol (2008, 83) is concerned that the introduction of patient choice would result in fixing 'the circumstances in which we make our choices' and 'the alternatives between which we may choose'. She does, however, point out that 'patient-customers' acquire relevant knowledge and become 'organised consumers' that 'help each other to choose' (Mol 2008, 17). While she hints at a possible role for a logic of consumer choice here, she does not explore this aspect much further, pointing out that healthcare professionals still are needed as go-betweens to e.g. educate patients how to correctly use the medical products they have chosen and obtained.

In our context of managing acute non-life-threatening situations, we found that for OTC and, surprisingly, also for POM, a logic of consumer choice was dominant among our informants. They were able and motivated to accumulate knowledge and build counter-expertise in order to exercise consumer choice, in several cases even eliminating the need for healthcare professionals as go-betweens. Many informants felt empowered to resist (openly or in the hidden) the marketing power of pharmaceutical companies or the medical authority of healthcare professionals. We observed how such consumer resistance fostered autonomous consumption practices such as self-diagnosis and self-treatment.

The formation of the autonomous consumer

We found that health information-seeking behaviours profoundly affect practices of medicine consumption through the extension of circumstances and ranges of consumer choice. Building upon Abercrombie's (1994) framework for consumer resistance, our findings demonstrate how access to alternative bodies of knowledge allows consumers to re-embody medical knowledge, build counter-expertise, and, thereby, become empowered to resist expert authority by 'challenging the decisions and knowledge of doctors in the medical encounter' (Lupton 1997, 97) and autonomously exercising consumer choice.

Re-embodying medical knowledges through a process of building counter-expertise seems to foster consumer resistance independent of whether consumers turn to knowledge based on their sensory perception, expert knowledges from online sources, or alternative knowledges promoted by peers, e.g. in online communities. While Jayanti & Singh (2010, 1080) question that 'social communities will enable consumers to acquire sufficient knowledge to engage in self-diagnosis or self-medication', our findings demonstrate that some consumers indeed feel empowered to autonomously engage in such practices of medicine consumption in acute non-life-threatening situations.

Furthermore, we found that many consumers go one step further in the construction of consumer choice, applying their re-embodied medical knowledge to create and maintain extensive medicine portfolios, often containing a variety of POMs. Consumers gain access to such medicine despite strict regulations, either by filling prescriptions but not actually applying the medicine as prescribed or, even more prevalent, by accessing less regulated markets. Rather than viewing 'the market as antagonistic networks of power designed to control and manipulate' (Denegri-Knott, Zwick, and Schroeder 2006), consumers creatively combine offerings from different markets to circumvent regulations. The enactment of consumer resistance by accessing global markets exemplifies how consumer resistance is 'informed and shaped by economic and cultural globalization' (Izberk-Bilgin 2010, 315).

Denegri-Knott et al. (2006, 962) argue that 'studying the practices of the self of consumers reveals the type of "self-regulating work" that consumers do when deliberating whether to consume or not' and that 'consumption is constructed as a site for

the care of the self'. In the spirit of 'Foucault's own insistence about the productive rather than repressive nature of power' (Lupton 1997, 101), consumer choice could thus be viewed as empowering individual consumers by providing them with the means to resist expert authority and marketing power as part of the care of the self. In Rose's (1999, 96) words: 'humans have become the kinds of creatures who can and do act upon themselves and against their limits, to increase their capacities and powers'.

The formation of the self-disciplined consumer

Shankar, Cherrier, and Canniford (2006, 1026) do not, however, see 'evidence of a shift in power relations further towards truly self-constituting subjects' in consumer empowerment, viewing the power of the market as totalising. They rather suggest that 'where there is empowerment there will also be disempowerment' and that consumer 'resistance merely supports domination'. We fundamentally agree with this critique of the liberal humanist notion of empowerment: while consumers may resist the medical expert knowledges they are directly confronted with – and experience the ensuing empowerment –, their health-related conduct in general, and their medicine consumption in particular, remain bio-politicised (Yngfalk 2016) by 'the guidelines and advice by which populations are surveyed, compared against norms, trained to conform to these norms and rendered productive' (Lupton 2013b, 116).

The medicalisation of everyday life has turned much of human existence into questions of personal health (Lupton 1997). Moralistic governance regimes (Askegaard et al. 2014) are attempting to create responsible consumer identities based on logics of choice, be it patient or consumer choice. In Rose's (1999) words: 'What began as a social norm here ends as a personal desire.' This responsibilisation is fuelled further by the market narratives portraying patients as consumers (Geiger and Finch 2016), which put 'pressure on today's citizens to see and think of themselves as consumers in the first instance' and 'as competent, creative, and responsible consumers' (Cova and Cova 2012).

National and supranational campaigns aimed at reducing the usage of antibiotics (Aabenhus et al. 2016) and other promotion of health messages by public and commercial agents have contributed to the rise of an ideology of healthism (Kristensen, Lim, and Askegaard 2016), which encourages consumers to prioritise personal health over all other

concerns. Our findings highlight how such an ideology contributes to the resistance towards expert authority, providing insight into how ideologies 'inform and shape consumer resistance' (Izberk-Bilgin 2010, 317). Moreover, we also exemplify how consumers navigate the tensions arising in case the demands placed by multiple ideologies clash.

Such resistance begets a shift away from ideals of responsible consumption based on filling prescriptions at the pharmacy and applying medicine according to instructions given by medical doctors or included in the package. Instead, we find ideals based on weighing 'risks over product benefits' (Delbaere and Smith 2007, 23). These authors have shown that fears of side-effects have a paralyzing effect on consumer choice as they incur the 'potential for consumers to cease taking medication'. Some consumers take this avoidance pattern to an extreme when they restrict their consumer choices to non-consumption based on an idea that all medicine represents an unnatural interference with the human body (Thompson and Troester 2002).

Our findings, thus, provide substantial empirical evidence for the ambivalence of consumer choice posited by prior research (Shankar, Cherrier, and Canniford 2006; Yngfalk 2016), demonstrating both its potential to empower as well as to discipline and paralyze. Health marketing initiatives of both public and commercial agents can be seen as an instance of 'governmentality', which connects 'technologies of power' to 'technologies of the self' (Foucault 1988). Here, the focus of disciplinary power is internalized, in the process forging 'self-disciplined consumers' (Shankar, Cherrier, and Canniford 2006).

Consumer choice in complex contexts of mixed commercial and public agencies

Our findings furthermore add an important aspect to the ambivalence of consumer choice by considering a more complex context of mixed commercial and public agencies. Choice is a double-edged sword, Shankar, Cherrier, and Canniford (2006) assert, with reference to Barry Schwartz's theorizing of the paradox of choice. However, in our context, this is true in a slightly different and additional sense. Making a wrong choice is obviously a significant risk when it comes to health care services. But making choices and advancing their logic may contribute to a shift in the institutional balance between

logics of care and choice in favour of the latter.

Obviously, some consumers may not consider this a problem since they belong to privileged classes that have both the cultural, social, and economic 'health capital' (Schneider-Kamp forthcoming) to exert their choices in 'privileged spaces' that allow transcending to some extent the 'imperatives and models of discipline imposed in a disciplinary sense' (Shankar, Cherrier, and Canniford 2006, 1019). Logics of choice may, therefore, increase inherent and existing inequalities. But choice in our case is not exclusively a right of the well-off. Thus, sticking to Mol's (2008) terminology, there is a real risk of simultaneously pursuing individual autonomy as a chooser and undermining one's role as a receiver of care.

This underlines the complexities involved in our context and points to what we see as one of our main contributive argument elements. "Future research could usefully examine contexts where actors who are less culturally stigmatized try to influence particular organizational fields" (Scaraboto and Fischer 2013, 1254). Arguably, our medicine consumers constitute exactly such a less stigmatized group. But even more importantly, Scaraboto and Fischer (2013) investigated a context where only commercial entities are the providing agents. Hence, any advance in market offerings can be seen as a victory of the henceforth empowered consumers. But it can conversely also be seen as a victory for (mainstream) marketers having enlarged their potential markets. In this sense, Scaraboto and Fischer's (2013) work does not really go beyond the liberal humanist notion of consumer empowerment.

Our investigation of a complex context with mixed commercial and public agencies therefore also in this manner complexifies the picture and exposes the ambiguity already present behind the immediate logics in Scaraboto and Fischer's (2013) narrative. Thus, our investigation fleshes out some of the consequences of the self-discipline enforced on consumers by contemporary governmentality, or, in other words, of Rose's (1999) paradox, "that not only are people expected to choose, but they are forced to choose in order to be 'free'" (Shankar et al. 2006 p. 1020) in a context, which is complexified by the contrasting of logics of care versus logics of choice.

Consumer autonomy and the implications for the healthcare market

The ambivalence of choice points to an underlying ambiguity of autonomy. Challenging standard bio-medical authority and pharmaceutical industries can be experienced as empowering but generates a risk of displacing authority to (other) commercial agents such as social media. Mol's (2008) logic of choice may generate consumer freedom to actively navigate alternative treatments, etc. whereas her logic of care may generate freedom from anxieties concerning whether one's treatment is optimal. There is autonomy in the freedom to choose and resist as well as in relying on the duties implicit in the Hippocratic oath (Askegaard 2009).

Our informants' efforts can be understood as a quest for consumer autonomy in the healthcare market. But the modern concept of autonomy is 'trapped between transcendence and immanence' (Bouchet 2007). Consumer autonomy in healthcare is both an outcome desired by some and an unavoidable by-product of the increasing focus on patient and consumer choice and the on-going marketisation of healthcare. This trend towards autonomy is concerning for at least two reasons. First, following Foucault's (1977) reflection on societal institutions, true autonomy in the face of technological and medical complexity has to remain an illusion. Consumer choice is inherently immersed in the health market and deeply dependent on its resources.

Second, and more importantly, there is a duality between increased consumer autonomy and the off-loading of the burden of treatment and other responsibilities in professional healthcare (Gusmano, Maschke, and Solomon 2019), i.e., consumers that lack the resources or willingness to act autonomously are anyway increasingly expected to do so (Schneider-Kamp and Askegaard 2020). As discussed above, this development has the potential to increase inherent and existing inequalities, obviously widening the gap between weak and resourceful healthcare consumers even further as e.g. demonstrated by Shim (2010) in her treatment on how cultural capital affects the interaction between healthcare consumers and healthcare professionals.

Future healthcare markets need to balance the needs of weak healthcare consumers unfit and unwilling to act autonomously with the desire for autonomy of the

resourceful ones. Clearly, ignoring the logic of consumer choice and returning to the paternalism of an exclusive focus on the logic of (professional) care is not a realistic option for the future of healthcare.

We choose to regard this issue as an outcome of the, for some perhaps paradoxical, difficulties of thinking autonomy in an age of individualism. The contemporary neo-liberal ethos of consumer culture is fundamentally based on a premise of individual autonomy. However, the irony is that autonomy cannot be an individual feature. The only way to secure autonomy as a political project is through a deep inscription in 'the social' since no one can maintain autonomy alone – individual 'autonomy' is either collective or based on the exploitation of others – and then it is no longer autonomy as a political project. As pointed out by Castoriadis (1987, 107), 'one cannot want autonomy without wanting it for everyone', hence 'its realization cannot be conceived of in its full scope except as a collective enterprise'.

While we agree with Jayanti and Singh (2010, 1080) that we need to exploit 'patient power' to 'reform medical markets so they serve society's best interest', we advocate a cautious approach that takes into account the ambivalent role of choice in consumer empowerment. Taking autonomy seriously means that only a differentiated approach can secure autonomy for both weak and resourceful health consumers. In other words, future healthcare markets need to concurrently explore the potential of and warrant against systemic negative effects of the pursuit of individual 'freedoms'.

Conclusion

We found that some consumers actively work towards expanding both the circumstances and the range of their choices based on access to online information as well as globalized markets, in the process becoming empowered to creatively circumvent regulations and to build counter-expertise to challenge and resist expert authority and marketing power. We demonstrated how consumer choice simultaneously forges self-disciplined consumers under the influence of moralistic governance regimes and ideologies.

This article, thus, provides empirical evidence for the ambivalent role of consumer choice in consumer empowerment and nuances how consumers navigate the

liminal space between expert authority and consumer autonomy, extending prior research by providing insights on the interrelation of consumer choice, consumer empowerment, and consumer resistance in a complex context of mixed commercial and public agencies. We finally related these results to the notion of autonomy and its ambiguity. Our results have implications for health marketers regarding how to take into account the complex interplay of consumer choice, consumer empowerment, and health inequality.

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